BASELINE → TREATMENT → FOLLOW-UP

Tests and milestones through treatment

Columns show sequence, not equal durations or appointments. A test is only complete when performed and reviewed. Echo timing and cancer-imaging timing are different. Symptoms can bring any check forward.

Check	Before starting	First cycles	Around week 12	Continuing / next phase	Longer-term plan
Blood tests	Baseline full blood count (FBC), renal and liver profile; dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) result; potassium / magnesium	Before each 21-day cycle; assess symptoms and oral dosing	Continue the required pre-dose checks	Use the selected active regimen's blood-test schedule	Investigate symptoms / persistent abnormalities
Echo / cardiac assessment	Baseline cardiac assessment	Earlier for symptoms / concern	Echo / multigated acquisition (heart pumping) scan (MUGA) due around 12 weeks	Continue every 12 weeks; keep the original clock	Agree follow-up for the active medicine and cardiac risk
electrocardiogram (ECG) / QT	Baseline ECG; correct K / Mg	Repeat 1–2 weeks after starting (cycle 1 Days 8–15)	Additional ECG if risk, interacting drugs or symptoms	Review QT-risk medicines and electrolytes	Individual ECG plan; early ECG is not an echo
Cancer imaging / markers	Baseline disease assessment	Clinical response; imaging if concern	Response review as planned; not tied to echo	Reassess disease and treatment plan	Imaging generally every 2–4 months; individual plan
Surgery / radiotherapy	Agree indication and sequence	No automatic concurrent radiation	Coordinate with both teams	Local treatment as indicated	Review recovery and late effects
Endocrine / bone health	Check hormone receptors / fracture risk	Plan endocrine therapy if indicated	dual-energy X-ray absorptiometry (bone density) (DEXA) when relevant to bone-risk plan	Endocrine timing by the treating team	Individual DEXA / bone follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00217 v7 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Lapatinib + capecitabine: the whole pathway

HER2-positive advanced breast cancer · previously treated

Lapatinib once daily on Days 1–21, continuously. Capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Repeat every 21 days while beneficial and tolerated. The capecitabine rest week is not a lapatinib rest week.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First cycles	Around week 12	Continuing / next phase	Longer-term plan
Cancer medicines NCCP	Confirm diagnosis and exact regimen	See the dose-day map: lapatinib daily; capecitabine Days 1–14	Echo due; treatment review	Review active medicines and benefit / tolerance	Review benefit / tolerance and next plan
Surgery / radiotherapy METASTATIC	Agree indication and sequence	No automatic concurrent radiation	Coordinate with both teams	Local treatment as indicated	Review recovery and late effects
Blood tests NCCP	Baseline FBC, renal and liver profile; DPD result; potassium / magnesium	Before each 21-day cycle; assess symptoms and oral dosing	Continue the required pre-dose checks	Use the selected active regimen's blood-test schedule	Investigate symptoms / persistent abnormalities
ECG / QT SMPC	Baseline ECG; correct K / Mg	Repeat 1–2 weeks after starting (cycle 1 Days 8–15)	Additional ECG if risk, interacting drugs or symptoms	Review QT-risk medicines and electrolytes	Individual ECG plan; early ECG is not an echo
Echo / cardiac checks NCCP · ESC	Baseline cardiac assessment	Earlier for symptoms / concern	Echo / MUGA due around 12 weeks	Continue every 12 weeks; keep the original clock	Agree follow-up for the active medicine and cardiac risk
Scans / tumour markers METASTATIC · MARKERS	Baseline disease assessment	Clinical response; imaging if concern	Response review as planned; not tied to echo	Reassess disease and treatment plan	Imaging generally every 2–4 months; individual plan

Care / check	Before starting	First cycles	Around week 12	Continuing / next phase	Longer-term plan
Endocrine / DEXA BONE	Check hormone receptors / fracture risk	Plan endocrine therapy if indicated	DEXA when relevant to bone-risk plan	Endocrine timing by the treating team	Individual DEXA / bone follow-up

What each check is for

1 Blood tests

FBC, kidney and liver profile at baseline and before every 21-day cycle. The team reviews the necessary results before a planned dose.

Check safety before the next dose.

Ask / act: Agree the booking owner, result reviewer and action if overdue.

Sources: NCCP

2 Heart tests

NCCP requires baseline left ventricular ejection fraction (heart pumping measurement) (LVEF) assessment in its cardiac precautions and echo/MUGA every 12 weeks (around cycle 5 if on time), or earlier for concern. EMA Tyverb specifies an ECG with QT before starting and again 1–2 weeks after starting, around Days 8–15 of cycle 1. Correct low potassium or magnesium before treatment and arrange extra ECG checks for interacting medicines, symptoms or QT risk. This early ECG is separate from the 12-week echo.

Monitor pumping function and relevant rhythm risk.

Ask / act: Agree the booking owner, result reviewer and action if overdue.

Sources: NCCP · SMPC

3 Cancer imaging

Record baseline disease sites. Response imaging is generally reviewed every 2–4 months under international guidance, adapted to symptoms, disease pace and treatment. The team selects the scan; this is not an automatic computed tomography (CT) chest/abdomen/pelvis order for every patient. Tumour markers may support assessment in selected cases, but do not alone decide progression or a treatment change.

Assess response or follow-up needs.

Ask / act: Agree the booking owner, result reviewer and action if overdue.

Sources: METASTATIC

4 Bone health

If the cancer is also hormone-receptor positive, endocrine treatment may form part of the wider plan. Bone-density testing is linked to aromatase inhibitors, ovarian suppression and fracture risk; it is not automatically required because a HER2 medicine is prescribed. Agree baseline DEXA where indicated and individual follow-up.

Assess fracture risk where relevant.

Ask / act: Agree the booking owner, result reviewer and action if overdue.

Sources: BONE

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-21 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.16